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infrastructuring reproductive labour in the maternity ward

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Epimethean design in practice: Infrastructuring reproductive labour in the maternity ward

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Abstract: This paper proposes an Epimethean mode of design that works with reproductive labour as a counterpoint to Anthropocentric narratives of progress. Rather than innovating new futures, design is approached as a practice of aftercare, sustaining endangered forms of life-making within existing institutions. Drawing on a long-term participatory design project at a large hospital maternity ward, the paper theorizes Epimethean design as a mode of infrastructuring that resists the translation of care into productivity and reclaims reproduction as a site of ontological and political contestation. At the same time, it argues that confronting reductionist ideas of productivity requires a double movement: undoing, by protecting fragile reproductive practices from capture within technosocial regimes, and doing, by cautiously reworking these regimes from within. This dual stance positions design as both an act of maintenance and a generative force, enabling reproductive labour to contest and redefine what counts as productive work.

Keywords: Epimethean design, reproductive labour, infrastructuring, participatory design

1. Introduction

This paper asks how we, as design researchers, might engage with the intertwined acts of doing and undoing in the post-Anthropocene. The question relates to a recurring tension in design research, namely, how to act critically and constructively through design when its practice so often predicated on anthropocentric projections that cast the world as an object to be innovated and mastered. Giaccardi and Redström (2020) address this question at a methodological level, describing it as a paradox embedded in contemporary design research agendas. While design increasingly operates within complex and distributed socio-technical conditions, its methods and tools remain primarily grounded in the human-centred and participatory design traditions. These traditions tend to privilege the local encounter as the primary site of design, often framed as a bounded set of interactions between users or

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participants and the artefact under development. As Giaccardi and Redström (2020) note, this methodological focus risks isolating design from the wider socio-technical networks through which such encounters are always already constituted. The project we report from in this paper may be said to epitomise this very paradox. It concerns a participatory design research initiative aimed at strengthening midwifery practices within a so-called in-house clinic, situated inside a large, technologized hospital maternity ward in Denmark. The concept of an in-house clinic refers to a clinic located inside a hospital, modelled on stand-alone maternity clinics outside hospitals.

Stand-alone clinics, whether private or public, are run by independent teams of midwives and provide care for uncomplicated pregnancies and births, including prenatal and postnatal services. As such, they delineate midwives' legally regulated sphere of autonomous practice from obstetric care, which denotes the specialized medical expertise provided by doctors. In the project discussed here, researchers employed participatory design tactics to establish a functioning in-house clinic that would allow midwives to practise midwifery as an autonomous, relational and embodied form of care within hospital premises. In many ways, the methods employed were quite conventional within human-centred and PD traditions and in this sense, the project might be read as a naïve attempt to hubristically improve the habitability of the world (Bonnet et al. 2022, p. 2).

However, as we will show and discuss, possible inroads for design to acquire an Epimethean mode of engaging with opaque socio-technical and institutional networks may still rely on familiar participatory design tools and methods oriented towards locally situated interactions and procedures. To explicate, we draw on the work of historian and political ecologist Stefania Barca (2020), who argues that any attempt to confront the ecological crisis must begin by developing a cogent notion of labour, one that repositions reproduction, rather than production, as the true condition of life.

Reproductive labour, in Barca's account, encompasses ecologies and earth care practices, maintenance, and renewal that sustain the world, yet remain obscured by the modern anthropocentric ideals of progress and productivity. Guided by Barca's proposition, the paper shows how participatory research tactics revealed that the data and metrics midwives were subjected to could be reappropriated as tools to support autonomous midwifery. Through collective infrastructuring efforts (Karasti, 2014; Pipek et al., 2017; Pipek & Wulf, 2009; Karasti & Syrjänen, 2004), combining everyday tinkering with collaborative design events (workshops), the issues and local interactions explored via design interventions (Halse & Boffi, 2016) gradually seeped into midwives' everyday practice and contributed to renewing the in-house clinic as a robust space for sustaining relational practices of care.

2. Rethinking design through Epimetheus

Invoking the figure of Epimetheus suggests that design might need to rethink its orientation. As a counterpoint to Promethean projection, Epimethean design emphasises aftercare, repair, and retrospection. This posture resonates with a number of scholars who, over the years, have sought to rehabilitate design from its compromised moral legacy and extractivist ideology. Among them are Fry and Dilnot's Manifesto for Redirective Design (2007), Fry's call for design after design (2017), Tonkinwise's design away (2013), Willis's account of design as

a reflective rather than projective practice (2014), and John Wood's framework for metadesign in the Anthropocene (2022). Similarly, Escobar's notion of ontological design (2018) highlights how design shapes not only artefacts but also the very worlds they bring into being, resonating with an Epimethean awareness of design's recursive entanglement with what has already been made. Latour's (2008) image of a "cautious Prometheus" in turn repositions the designer as one who proceeds carefully, aware of design's unintended consequences. Here the Epimethean inflection might be read as a subtle radicalisation of Latour's vision for design, suggesting that caution alone is not enough and that designers must place even greater emphasis on retrospection and attend to the consequences of previous actions.

Granted, Latour's thought experiment presented design in a rather flattering light, endowing it with greater ethical potential than architecture or engineering. Yet one might still ask whether a heightened retrospective orientation makes design's practical imperatives any clearer. Writing from the perspective of architecture, Ardeth (2023) employs Epimetheus to question the very scope and authority of architectural practice. How, for instance, might we understand the relation between the built environment, energy transitions, net-zero cities, and the redistribution of resources? Such questions presuppose a level of agency that extends far beyond the reach of architectural and planning practices. Similar questions can, and perhaps should, be asked with design practices in mind. What can we, as researchers, realistically expect from design? The question has a double edge. It can be directed critically at those moments when design claims the capacity to solve complex problems at almost any scale, something that, unfortunately, is far from rare. But it also turns inward, toward design research itself, where calls for reflexivity and critical awareness may risk becoming detached from the situated conditions under which design operates. To design with Epimetheus, then, we suggest, is not only to reflect on design's moral and material consequences in hindsight, but to acknowledge the limited scope of design's agency, and, despite these limitations, to insist on acting with care from within rather than above the networks and constraints that shape our collective conditions of action.

2.1 Reproductive Labour in the Anthropocene

The Anthropocene narrative presents its protagonist, the "anthropos", as a gender-neutral human agent, thereby concealing the patriarchal order that sustains industrial modernity. As feminist theorists such as Federici (2009), Fraser (2016), and STS scholar Puig de la Bellacasa (2017) have shown, capitalism's capacity to expand has always depended on the unpaid, feminised labour of reproduction, domestic labour, and ecological maintenance that sustain human and nonhuman life but remain excluded from systems of value.

Barca (2020) names these as the forces of reproduction, the collective energies that make life possible in the face of extractive destruction. Their invisibility in the Anthropocene narrative mirrors their historical marginalisation in capitalist modernity. By re-centring these forces, Barca reframes the ecological crisis not as a failure of technology but as a crisis of care and a systemic exhaustion of reproductive capacities at every scale, from the body to the biosphere. Zooming in from theorizing the Anthropocene and planetary crisis to birthgiving, arguably the most anthropocentric act of labour, we propose that design research must concern itself not with accelerating managerial efficiency or innovation but with sustaining the fragile infrastructures of care, repair, and maintenance on which both

social and ecological life depend. It must learn to work in the intervals between breakdown and renewal, and to engage in reproduction as an active, collective practice rather than a passive background to production. Seen through an Epimethean lens, the care crisis emerges as an issue for design to grapple with, not only because infrastructures, technologies, and policies shape the delivery of care but because they encode and reproduce specific ontologies of dependency, temporality, and value. Consider, for instance, the design of maternity wards, spaces that materialise the translation of reproductive labour into measurable productivity. Every architectural layout, digital monitor, and procedural protocol participates in an infrastructural imaginary of care as efficiency, control, and throughput. What is at stake, therefore, is not merely how care is organised but what kinds of relations are made possible or foreclosed through its design.

2.2 Epimethian design as a research practise

Our primary concern in this paper is how we, as design researchers, may position ourselves to better attend to the intertwined acts of making and unmaking within a post-anthropocentric condition. Raising these questions in the context of a technologically saturated and tightly managed hospital setting brings us face to face with the paradox articulated by Giaccardi and Redström (2021). Relying on PD tactics and methods grounded in human-centred perspectives that privilege local interactions as primary sites of design, we have no way of knowing in advance how staged collaborative activities will connect to the wider network of human and non-human actors that exert pressure on midwives' autonomous practice.

The Epimethian approach we present here emphasises the slow, cumulative labour of maintaining continuity across a network of events while attempting to forge change within a system that constantly risks collapsing back into habitual logics. In the in-house-clinic project, PD researchers worked closely with midwifery management and midwives to formulate and stage concrete collaborative initiatives. They were able to do so by relying on their independent research time, which allowed them to engage with pertinent issues at the hospital ward outside the temporal constraints of external project cycles and funding regimes. This independence turned out to be essential, as more than a year and a half passed before it was even possible to stage the first collaborative workshop. Organisational restructuring and a sudden hiring freeze meant that, for long periods, the researchers could do little more than carry out traditional forms of ethnographic fieldwork.

The underlying logic of an Epimethian approach rests on an understanding of design as an everyday making activity (Manzini, 1992; Manzini and Cullars, 2015), something that, in principle, takes place everywhere and all the time across an organisation (Bjørn and Boulus-Rødje, 2015). Design, in this sense, is not confined to the staged local encounters that workshops represent but unfolds continuously within and beyond them. Yet in the everyday reality of a busy maternity ward, where time is scarce and opportunities for collective acts of making are almost non-existent, collaborative explorations may become central to reworking issues and practices precisely because, in the absence of such occasions in daily work, they offer rare moments to jointly explore, reflect, and experiment with alternative ways of doing things together.

3. The In-house Clinic Project: Infrastructuring Reproductive Labour

The in-house clinic project is a long-term and still ongoing participatory design collaboration between one of the authors, her research colleague, and local managers and midwives at a large regional hospital maternity ward in the metropolitan area of Copenhagen, Denmark. As such, the project should be understood in the context of the Danish healthcare system, characterised by public tax-funded universal coverage. The collaboration began in 2022, following an invitation from the ward's chief midwife, who, together with her colleagues, was searching for alternative ways to counter what they experienced as increasing pressure on midwives' professional autonomy within the hospital sector.

The ward's in-house clinic became the obvious focal point, as the clinic, implemented three years earlier, had been established to strengthen care for women seeking uncomplicated births without medical intervention. The clinic, since its implementation, had struggled to gain a stable foothold within the hospital, and had not yet succeeded in operating as an autonomous and robust platform for uncomplicated births. It was used only sporadically by a small group of midwives who were particularly eager to practise at the clinic. For local midwifery management it was difficult to practically support this group of midwives, because the maternity ward suffered from constant staff shortages. The conventional part of the labour ward was equipped to handle all types of births, from highly complex to the uncomplicated, and amid persistent staff shortages, the in-house clinic had come to operate mainly as a flexible extension of the conventional ward, rather than as a dedicated space for normal, low-risk births.

3.1 Contrasting Modes of Midwifery

What is essential to understand in this context is that midwifery at the in-house clinic differs profoundly from midwifery at the conventional labour ward. In the clinic, a midwife typically works alone and stays with the woman in labour from admission to delivery. The work centres on building a trusting relationship with the woman and her companions, to support the woman through often long periods of intense pain, and to encourage her to rely on her body's own rhythm and capacity to give birth. Reproductive labour like this requires extensive experience and training grounded in relational and embodied forms of care.

By contrast, in the conventional labour ward, midwifery becomes a far more fragmented and task-oriented practice. Here, health staff, including nurses, healthcare assistants, and obstetricians, can oversee several births at once using monitoring technologies and various medical interventions. A midwife may complete one delivery, then attend to another woman still in early labour, check on someone whose contractions have stalled, and assist elsewhere if an emergency arises. In this setting, midwifery expertise becomes distributed across multiple assisting tasks, unfolding in different rooms and detached from the birthing woman's bodily rhythms. As a result, the conditions for a midwife to remain continuously present with any single woman are greatly reduced, and care work takes on a fundamentally different form.

3. 2 Organisational Logics and everyday routines at the Ward

To understand the conditions of midwifery in the hospital, it is important to stress that the maternity ward is a tightly managed space where the imperative of shift coverage overrides

everything else. Rotas are planned months in advance by a central scheduling unit to ensure uninterrupted staffing, and each midwife follows an individual schedule balancing clinical shifts with duties such as home-birth visits, supervision, consultations or part-time arrangements.

When a midwife arrives for a shift, she usually has no way of knowing which colleagues she will be working with or what tasks she will be assigned to, as these are distributed by the supervising midwife on arrival. It is not unusual to go months without encountering the same colleague, and the daily routine leaves no time for collective reflection or debriefing. This becomes particularly evident for midwives working in the clinic.

The clinic is situated at the far end of the ward, well away from the main unit. When a midwife leaves the clinic after completing a birth, she will typically continue directly into ongoing tasks at the main ward, joining a team already absorbed in their own immediate pressures and unaware of what has just unfolded at the clinic.

4. Tracing the in-house-clinic project

The following account traces how the in-house clinic project evolved over a year of collaborative experimentation in 2024. Moving between workshops and everyday practice, we describe how design activities that focused on local routines and socio-technical arrangements created unexpected openings through which forms of data and coordination once used to monitor productivity became drivers for sustaining the reproductive labour of midwifery in the clinic.

4.1 Probing engagement through collective explorations

The first workshop in the in-house clinic project was primarily concerned with exploring whether midwives with an interest in the clinic could be mobilised around the project. After more than a year and a half of fieldwork, conversations, interviews and several setbacks along the way, there was still no prospect of dedicated resources being allocated to the clinic. Many of the midwives interviewed during the initial fieldwork had voiced serious doubts about whether the clinic could ever function without such dedicated support. At the same time, there was a pronounced sense of project fatigue at the ward, as previous initiatives, including the in-house clinic itself, had often been thwarted due to shifting priorities.

Nonetheless, the Deputy Chief Midwife (DCMW), through careful coordination and months of tinkering with shift plans, managed to book eight midwives for a workshop in late January 2024. In preparation for the event, the PD researchers had selected ethnographic excerpts from fieldwork, including brief quotes capturing reflections from the midwives they had spoken to. From the outset of the session, the DCMW was candid about the lack of resources, explaining that while local management was striving to secure support, they could not guarantee anything, as decisions from higher up could override their efforts.

During the first part of the workshop, midwives were asked to select quotes they felt particularly resonated with and identify areas that needed attention in order to strengthen the clinic. In the second part, the PD researchers employed scenario techniques, akin to user

journeys, to stimulate critical reflection and debate about what an ideal birth process in the clinic might look like in the future, from admission to discharge.



Figure 1 Midwives debating future scenarios for the clinic.

The point of all this was not to define or design an ideal pathway, which would not have made much sense in a situation where the conditions for reviving the clinic were far from ideal. Everyone knew there were not enough resources to sustain a dedicated team in the clinic.

At this stage in the process, amid considerable uncertainty about the future direction of the project, there was a pronounced need for a shared reference point through which abstract visions could be grounded in concrete exchanges about midwifery in the clinic, allowing midwives to articulate why relational and embodied forms of care mattered to them.

In the wrap-up, the midwives expressed enthusiasm for the project, although some were more cautious than others about its prospects. Most found it difficult to imagine how the clinic would ever come to function without a dedicated team. The DCMW, for her part, committed to organising a special scheduling arrangement that would allow interested midwives to spend more time in the clinic while still covering other essential duties such as consultations and supervision.

4. 2 Everyday tinkering and rehearsing shift planning

During the following period, the DCMW developed a small, hands-on system to increase midwives' opportunities to take shifts in the clinic. Shift coverage at the ward was managed centrally by a planning unit through recurring rotation set months in advance. As a workaround, the DCMW introduced a "C" (for clinic) next to the names of participants from the first workshop and other midwives interested in the clinic. These lists were then handed to supervising midwives responsible for distributing tasks among those on duty. In addition, the DCMW checked in regularly with the supervisors to encourage them to prioritise the "C-marked" midwives for clinic births whenever possible.

Over the next months, activity at the clinic gradually increased, though coverage remained incomplete and no new resources were allocated. At this stage in the process, the most powerful leverage point for improving conditions on the ground appeared to lie in the ongoing, ad hoc tinkering with shift coverage, which, in turn, provided the basis for the second workshop.

For this workshop the PD researchers, the DCMW and one of the ward's central shift planners co-invented an "as-if" scenario exercise, focused on shift coverage in the clinic. The

workshop brought together eleven midwives. More than half had taken part in the first event, while five were new to the project. In the workshop, midwives were handed out short quotes from the first design event to bring the newcomers on board and then invited to discuss key priorities for strengthening collaboration in the clinic.

After sharing in plenary, the DCMW introduced the main exercise. She emphasised that this was a try-out, to explore what could be achieved with the resources currently available. She also stressed that shifts allocated during the workshop would form a blueprint for an in-situ full scale rehearsal of at the clinic over four weeks in September. The shift planner, who had been invited to help co-facilitate the workshop, went on to outline key regulations in the midwives' collective union agreement, including shift lengths and mandatory breaks. Working on A3 sheets with only the weekend shifts pre-assigned, midwives were now asked to negotiate and distribute the remaining shifts among themselves to ensure, as far as possible, full coverage of the clinic during the upcoming full-scale trial.

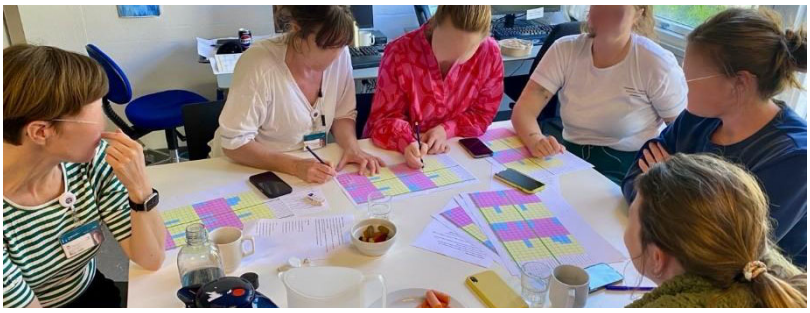


Figure 2 Midwives negotiating the distribution of shifts for the September trial.

Balancing other duties such as consultations, supervision, and training with a commitment to sustain a functional clinical environment quickly led to exchanges about how much time one needed to spend in the clinic to stay attuned to its everyday practices. It also prompted discussions about how less experienced midwives could be supported and how to ensure smooth handovers between shifts.

During these conversations, it became evident how much the midwives depended on one another if the clinic were to function as a midwife-led clinic, and that everyone needed to stay informed about what was happening at the clinic while on duty elsewhere. In the debriefing after the exercise, this led to the appointment of a temporary team coordinator, who was tasked with reporting regularly on developments at the clinic through the midwives' IT-platform. She was also encouraged to closely follow the upcoming trial and come up with suggestions for evaluating the September experiment.

4. 3 Doing and undoing evaluation

More than three months later, the full-scale trial was launched. Based on the blueprint developed in the previous workshops, and through close coordination between the DCMW and the central shift planner, midwives who had participated in the two previous workshops took turns covering the clinic across day, evening, and night shifts, to the fullest extent possible within existing constraints.

During the trial period, the DCMW continued to check in with supervising midwives on duty, reminding them to assign newly admitted women eligible for low-risk births to the clinic

whenever possible. Meanwhile, the new team coordinator provided regular updates on the clinic's activities and decided to compare the number of births during the trial with the same period the previous year, to find a way to evaluate the trial. To everyone's surprise, she found a 50 percent increase in completed births in the clinic. In the larger scheme of the hospital's governance structures, this was quite significant.

Since the clinic's introduction in 2019, its low number of completed births had remained the main obstacle to securing resources for a dedicated team. To verify this data, the coordinator chose to manually review all patient records from the trial period only to discover that two latency rooms for women awaiting initial assessment had been mistakenly registered as part of the clinic. This had caused an overcount of transfers from the clinic to the main ward. Transfers, in turn, had long been a charged issue, as previous criticism of the clinic had centred on safety and the risk of too many women being transferred to the main ward for medical intervention. In the hospital's IT system, births were only registered as completed in the clinic if the entire process took place there, meaning that even women transferred just before delivery were recorded as main ward births.

All these new discoveries prompted the PD researchers, DCMW and team coordinator to reconsider the focus of the upcoming third workshop. They decided to have the team coordinator present her findings at the beginning of the session, and therefore workshop explorations came to revolve around how transfer decisions were made and registered, rather than strictly evaluating the September trial. Transferring a woman from the clinic to the ward was always a matter of individual professional judgement. Making that call could be challenging especially for midwives with less experience and in borderline cases where there was no obvious reason to transfer other than a prolonged labour. In the wrap-up it was decided that the team coordinator would continue monitoring patient records.



Figure 3 Midwives discussing the registration of birth numbers and transfer decisions are made.

In the months that followed, the issue of transfers remained a recurring theme. Midwives began reporting back on individual transfer decisions, and new guidelines for handling transfers in the clinic were produced in subsequent workshops. Among these was a new rule stating that whenever a midwife was in doubt about a transfer, consulting a supervisor by phone was no longer considered sufficient. Instead, the supervisor was now required to physically come down to the clinic and assess the situation directly in the delivery room.

By March 2025, a new four-week trial was conducted, this time with two midwives covering day, evening, and night shifts in the clinic. This model has since been fully implemented, and the number of completed births in the clinic continues to rise.

5. Doings and undoings in the Maternity Ward

With the above account, the aim has been to emphasise the tactical and collective infrastructuring efforts distributed across researchers, managers, and midwives. The relations that emerge through such a process cannot be known in advance, they only become visible as the work proceeds: Had the DCMW's initial initiative after the first workshop not led more midwives to take shifts in the clinic, it would hardly have made sense to attempt the shift-planning exercise in the following workshop. The as-if scheduling exercise in the second workshop would have remained merely symbolic unless the plan had later been tested during the September trial. Likewise, had the new team coordinator not been tasked with devising a format for evaluation, she would not have gone through all the medical records manually, and the registration errors in the system would have gone unnoticed. And similarly, there would not have been an obvious way to collectively respond to these findings, unless a group of midwives had already established a shared commitment to strengthen their clinic.

Although the PD tactics presented here largely privilege the staged collaborative events themselves and are organised around a bounded series of interactions between participants (midwives) and the artefact under development (the in-house clinic), the paradox articulated by Giaccardi and Redström (2021) remains fully present in the in-house-clinic project. It takes shape as a collective effort to navigate between shift planning, registration systems, data infrastructures, protocols of safety and productivity, and competing versions of what counts as maternity care. With Epimetheus in mind, however, the question is not whether this paradox can be resolved, but how we might learn to work within it, and to accept it as a condition of design research rather than an obstacle to be overcome.

The in-house clinic case extends Barca's feminist critique of the Anthropocene by showing how empowering reproductive labour through design itself becomes an endeavour that encompasses not only care and maintenance but also the organisational and epistemic labour required to keep such care viable within technocratic infrastructures.

Reproductive labour in design research is not only the object of care (the midwives' work) but also the mode of research itself. The researchers' efforts to sustain continuity across workshops, negotiate with scheduling systems, and protect fragile forms of midwifery practice constitute reproductive labour in their own right. They work to preserve spaces of relational care against institutional erosion, performing a kind of academic and organisational midwifery. By repositioning participatory design as a form of reproductive labour, the project reframes the design researcher's task from producing new artefacts to accompanying and sustaining ongoing processes of world-making.

This reconfiguration applies to reproductive labour in Barca's sense (2020), albeit at a vastly different scale. Labour here appears in multiple, interwoven registers: labour as giving birth, the embodied and relational practice of midwives attending to women and infants; labour as midwifery, the professional and organisational care that keeps these practices alive within an extractive institution; and labour as participatory design, a mode of collective afterthought that attends to the reproduction of care systems rather than their optimisation. In each case, labour is oriented toward renewal through continuity rather than progress and production.

Such a view positions Epimethean design as a form of aftercare for the Anthropocene: a practice that does not seek to design the future but to accompany the present's ongoing labours of repair. The figure of the designer researcher thus converges with that of the midwife—both are called to work with what exceeds their control, to sense the rhythms of a process already underway, and to intervene only insofar as it supports the emergence of life. The clinic project demonstrates how this alignment can materialise through seemingly modest acts - adjusting shift plans, redefining evaluation criteria, sustaining meetings - that nonetheless reconfigure the infrastructural conditions of care.

6. Concluding remarks

In reimagining participatory design as a reproductive practice, the project applies a feminist ecological critique of the Anthropocene to an understanding of labour in a maternity ward. It suggests that the sustainable institutional renewal required today cannot be achieved through Promethean innovation but through Epimethean practices of accompaniment, maintenance, and reflection. To engage design as reproductive labour is to acknowledge that its most consequential work may lie not in inventing new worlds but in sustaining those already struggling to persist.

As we have attempted to show, adopting an Epimethean stance in participatory design research also requires us to question the anthropocentric narratives within even the most human-centred of endeavours - such as organising reproductive labour in the maternity ward. This case demonstrates that even here, where care for human life seems primary, design must attend to the entangled ecologies of human and more-than-human actors, material systems, and institutional logics that co-constitute conditions of care.

Accordingly, we contend that Epimethean design should engage these multiple entities, to reclaim reproductive labour as a site of ontological and political contestation. By doing so, participatory design research can confront the cultural and material hierarchies that separate care from value, and reproduction from production. Ultimately, this orientation invites a design research attuned to the long temporalities of maintenance and care, where intervention becomes a practice of stewardship and afterthought - an ongoing contribution to the collective reproduction of life in damaged but still liveable worlds.

As such, the project also adds to ongoing discussions in PD about how design researchers can work within, yet also move beyond, rigid project formats and external funding schemes. Further research under the auspices of what we here have termed Epimethean design may point to the development of methods that aim to extend participatory approaches to better encompass the long duree and emphasis of regenerative labour over change in project collaborations.

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